



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : نوال شعبان مخلوف نجم

العنوان : الدرب الاحمر الاحمر
القاهرة

الحي / المدينة : الحسين

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 24-09-2026

رقم البطاقة : 25105162300441 رقم التليفون : 01064040391

العمر (تاريخ الميلاد) : 16-05-1951 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: نوال شعبان مخلوف نجم National ID: 25105162300441	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



نوال
شعبان مخلوف نجم
٣ ش - سكة الشابورى - الحليمه الجديده
الدرب الاحمر - القاهره

١٩٥١/٠٥/١٦ ٢٥١.٥١٦٢٣.٠٠٤٤١
I19392973

٢٠٢٠/٠٢ ٢٥١.٥١٦٢٣.٠٠٤٤١



ربة منزل
أنثى
مسلمة
متزوجة
عبد الهادى عنوانى عامر
البطاقة مارية حتى ٢٠٢٧/٠٢/١٦



١





مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الست المصرية
صحة
مصر

MDT FORM

16 / 4 / 20 25 G 1130
500

Name	نوال شعبان خليل	MRN	٨٣٥٧٠٨٢٨
Age	٧٣	ID	
Menopausal status		Phone	٠١٠٦٤٠٤٠٣٩١
NRS SCORE		Marital status	
BMI		Family history	٠١١٥٠٨٠٠٩٠٩
CO morbidity	HTN	REASON OF MDT	

Clinical Data

BX:

ER:

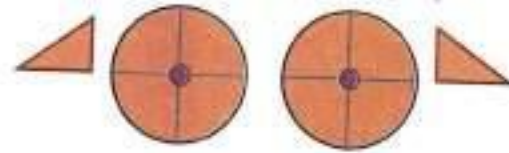
PR:

HER 2:

Ki67:

R

L



T N M

Sonomammography:

Block revision ⇒ (R)

Mets Workup:

73 Yold Female,
MBC to bone ER
PR +ve

Recommendations:

HER2 -ve by IHC on At since 2/2-23

CTCAP 13/4/2025 ⇒ status f.u showing ⇒ Progressive disease.

Pt shifted to Faskolex 14/4/2025 ⇒ for possibility
of adding CDK 4/6 inh.

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name	: الأستاذ / نوال شعبان مخلوف نجم	Request No.	: 13007543
Patient No.	: 25105162300441	Request Date	: 16/04/2025
Path No.	: SR-593/25	Verification Date	: 27/04/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 73 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.

Nature of Specimen: Slide Revision.

Gross Examination:

- 1 paraffin block labeled by (769/23) was received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1).
- * DCIS: No DCIS.
- * Angiolinvasion: No.
- * Perineural invasion: Positive.
- * IPT:
- ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8 /8).
- PR: Strong nuclear reaction in 90% of tumor cells (Allred score 8 /8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 8% nuclear reaction of tumor cells.

Diagnosis:

Left breast cancer, Slide revision:

Invasive duct carcinoma G II with positive perineural invasion.

IPT:

- ER: Positive (score 8 /8).
- PR: Positive (score 8 /8).
- Her2: Negative (score 0).
- Ki67: Positive in 8% of tumor cells

Signature

Verified by :

DR.Menna Nabil

Printing Date : 30/04/2025 09:53:23

Page 1 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذة / نوال شعبان مخلوف نجم	Request No.	: 13007543
Patient No.	: 25105162300441	Request Date	: 16/04/2025
Path No.	: SR-593/25	Verification Date	: 27/04/2025
Sex	: Female	Financial Category	: ابورام دار السلام - نفقة الدولة
Age	: 73 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Prof. Dr. Lubna Omar.
Dr. Heba Hassan,

Verified by :
DR.Menna Nabil

Printing Date : 30/04/2025 09:53:23

Page 2 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No, (4) - First Floor-Building No, 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

٤٧.١

PATIENT NAME: NAWAL SHABAN MAKHLOF NEGEM

GENDER: Female

DATE: 26-Apr-2026 11:51 AM

PATIENT ID: 2023216380

AGE: 74 y

ORDER: 3812520,3812521

FOLLOW-UP POST- CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

Compared to the previous study dated 09/09/2025, The current study shows :

- CHEST

- **Rather, stationary course regrading:**

1. Left breast mass still measuring about 3.1 x 2.5 cm reaching the skin and pectoral muscle.
 2. The size and number of the left axillary lymph nodes reaching 10 x 7 mm in axial dimensions.
 3. Bilateral supraclavicular subcentimetric lymph nodes.
 4. Prominent pre-carinal and prevascular lymph nodes; the largest is the former measuring about 10 mm in its maximum short axis.
 5. Middle lobar lateral segment and fissural pulmonary nodules, the largest measuring 6.5 mm.
- No pulmonary masses, areas of consolidation or cavitary lesions.
 - No evidence of pleural effusion, pleural thickening or masses seen.
 - Mild cardiomegaly. Prominent main pulmonary artery measuring about 3.2 cm in diameter for echocardiography assessment.
 - No pericardial effusion detected.
 - Atherosclerosis calcification of the aorta.
 - Hiatus hernia is noted.

2- ABDOMEN AND PELVIS:

- The liver is of enlarged size with low fatty homogenous CT texture. No focal lesions or biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main tributaries.
- Normal CT appearance of the pancreas, spleen, kidneys (apart from right renal lower calyceal small stone), adrenals, aorta (apart from atheromatous calcification), IVC and rest of pelvic organs.

PATIENT: NAWAL SHABAN MAKHLOF NEGEM, 2023216380, Female, 74 y

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Report version 2, 30-Apr-2026 12:47 PM



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مركز بهية الهرم
لاشعة علوية ليل شرح الهرم بجوار مستشفى الجيزة
عندما في بويته الشيوخ واد
٤- في الاول - الشيخ زايد امام كلية الهندسة

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- No abdominal or pelvic collections.
- Small fat containing umbilical hernia is noted.

revised bone window revealed:

- **Mild sclerotic transformation of the right 5th rib posterolateral aspect, D4, D7, L3, right iliac and right posterior acetabulum mixed lytic and sclerotic lesions with increased extent of some of the other eg at L2 vertebra.**
- **Stable:**
 1. Other wide spread osseous lesions involving the axial and appendicular skeleton.
 2. Sternal lytic lesion with cortical disruption and thin soft tissue sheet
 3. L3 vertebral body mild anterior wedging is still noted.
 4. Bilateral multilevel rib fractures.
 5. Spondylo-degenerative changes.

OPINION:

status follow up showing:

- **Mild sclerotic transformation of the some of the osseous lesions as described. For correlation with bone scan findings.**
- **Stable course of the rest of the study.**

Reported By,
Dr. Ali Saber, MSc
Dr. Mahmoud Ibrahim, MD

30-Apr-2026 12:47 PM



PATIENT NAME: NAWAL SHABAN MAKHLOF NEGEM

GENDER: Female

DATE : 12-May-2025 10:13 AM

PATIENT ID: 2023216380

AGE: 73 y

ORDER: 3352300

99MTC-MDP BONE SCAN**Desired Information: -**
skeletal survey.**Technique:**

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP :

Report:

Compared to previous bone scan dated 11/9/2024, the current study revealed more or less stationary course of the previously seen metastatic lesions which are seen at skull bones, sternum, multiple bilateral ribs, spine, pelvic bones, both shoulder and both hip regions for correlation with clinical and other radiological data.
Degenerative changes are seen over dorso-lumbar vertebrae.
Arthritic changes are seen over both knees.
Both kidneys are normally visualized.

Conclusion:

Follow up bone scan shows rather stationary course of the metastatic osseous deposits, compared to 11/9/2024.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant

12-May-2025 11:55 AM

PATIENT: NAWAL SHABAN MAKHLOF NEGEM, 2023216380, Female, 73 y

Report version 1, 12-May-2025 11:55 AM

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PATIENT NAME: NAWAL SHABAN MAKHLOF NEGEM

GENDER: Female

DATE : 25-Aug-2026 10:50 AM

FOLLOW-UP NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

**** Compared to the previous study dated 26/04/2026, The current study shows :**

1-CHEST

- **No cross interval changes regarding:**

- No gross mediastinal enlargement.
- 1. Left breast mass still measuring about 3.1 x 2.5 cm reaching the skin and pectoral muscle.
- 2. The size and number of the left axillary lymph nodes reaching 10 x 7 mm in axial dimensions.
- 3. Bilateral supraclavicular subcentimetric lymph nodes.
- 4. Prominent pre-carinal and prevascular lymph nodes; the largest is the former measuring about 10 mm in its maximum short axis.
- 5. Middle lobar lateral segment and fissural pulmonary nodules, the largest measuring 6.5 mm.
- No pulmonary masses, areas of consolidation or cavity lesions.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Mild cardiomegaly. Prominent main pulmonary artery measuring about 3.2 cm in diameter for echocardiography assessment.
- Ecstasic ascending aorta measuring 3.8 cm.
- No pericardial effusion detected.
- Atherosclerosis calcification of the aorta.
- Still seen small hiatus hernia

2-ABDOMEN AND PELVIS:

- The liver is of enlarged size with slightly low homogenous CT texture.
- No focal lesions or biliary radicles dilatation.
- Normal CT appearance of the pancreas, spleen, kidneys (apart from right renal lower calyceal small stone), adrenals and rest of pelvic organs.

PATIENT: NAWAL SHABAN MAKHLOF NEGEM, 2023216380, Female, 75 y

Report version 2, 25-Aug-2026 03:09 PM

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- Aortiliac atheromatous calcification.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- No abdominal or pelvic collections.
- Small fat containing umbilical and right inguinal hernias are still noted.

The revised bone window revealed:

- **Rather, stationary course of the previously seen of the wide spread predominantly sclerotic osseous lesions involving the axial and appendicular skeleton.**
- **Still noted unchanged:**
 1. sternal lytic lesion with cortical disruption and thin soft tissue sheet
 2. L3 vertebral body mild anterior wedging.
 3. Multiple bilateral rib fractures , of different chronological ages
 4. Spondylo-degenerative changes.

OPINION:

Status follow up showing:

- **Stationary course regarding:**

1. **Left breast malignant mass lesion.**
2. **Wide spread predominantly sclerotic osseous metastases.**
- **No newly developed lesions.**

Reported By,

Dr. Sally Mahmoud, MSc

Dr. Hekmat Samir, MD

25-Aug-2026 03:09 PM

PATIENT: NAWAL SHABAN MAKHLOF NEGEM, 2023216360, Female, 75 y

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مركز بحوث
البحر الأبيض المتوسط
مستشفى بحوث البحيرة
البحر الأبيض المتوسط
مركز بحوث البحيرة
مستشفى بحوث البحيرة
مركز بحوث البحيرة
مستشفى بحوث البحيرة

BQHEYA

Radiology Department

بن هية
قسم الأشعة العظمية

PATIENT NAME: NAWAL SHABAN MAKHLOF NEGEM

GENDER: Female

DATE : 03-May-2026 11:32 AM

PATIENT ID: 2023216380

AGE: 74 y

ORDER: 3853993

99mTc-MDP BONE SCAN**Desired Information: -**
skeletal survey.**Technique:**
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:**Report:**
Compared to previous bone scan dated 10/1/2025, the current study revealed more or less stationary course of the previously seen metastatic bone deposits with more sclerotic nature of some of the lesions due to therapy effect.
Degenerative changes are seen over dorso-lumbar vertebrae.
Arthritic changes are seen over both knees.
Both kidneys are normally visualized.**Conclusion:****More or less stationary course.****Reported By,****Prof. Huda Fathy, Nuclear Medicine Consultant****03-May-2026 04:50 PM**

PATIENT: NAWAL SHABAN MAKHLOF NEGEM, 2023216380, Female, 74 y

Report version 1, 03-May-2026 04:50 PM

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طبيب عيادة العظام
أستاذ مشارك كلية الطب جامعة القاهرة، القاهرة
مستشفى عيادة الشيخ زايد
10، حي النور، الشيخ زايد، القاهرة الجديدة



PATIENT NAME: NAWAL SHABAN MAKHLOF NEGEM

PATIENT ID: 2023216380

GENDER: Female

AGE: 75 y

DATE : 25-Aug-2026 10:50 AM

ORDER: 3973348.4010376.4010377

FOLLOW-UP NON-CONTRAST MULTISLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

** Compared to the previous study dated 26/04/2026, The current study shows :

1- CHEST

No gross interval changes regrading:

1. Left breast mass still measuring about 3.1 x 2.5 cm reaching the skin and pectoral muscle.
2. The size and number of the left axillary lymph nodes reaching 10 x 7 mm in axial dimensions.
3. Bilateral supraclavicular subcentimetric lymph nodes.
4. Prominent pre-carinal and prevascular lymph nodes; the largest is the former measuring about 10 mm in its maximum short axis.
5. Middle lobar lateral segment and fissural pulmonary nodules, the largest measuring 6.5 mm.
- No pulmonary masses, areas of consolidation or cavitary lesions.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Mild cardiomegaly. Prominent main pulmonary artery measuring about 3.2 cm in diameter for echocardiography assessment.
- Ectatic ascending aorta measuring 3.8 cm.
- No pericardial effusion detected.
- Atherosclerosis calcification of the aorta.
- Still seen small hiatus hernia

2- ABDOMEN AND PELVIS:

- The liver is of enlarged size with slightly low homogenous CT texture.
- No focal lesions or biliary radicles dilatation.
- Normal CT appearance of the pancreas, spleen, kidneys (apart from right renal lower calyceal small stone), adrenals and rest of pelvic organs.

PATIENT: NAWAL SHABAN MAKHLOF NEGEM, 2023216380, Female, 75 y

Report version 2, 25-Aug-2026 03:09 PM

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مركز بهرية الأشعة
مستشفى عين شمس الجديع
مستشفى عين شمس الجديع
مستشفى عين شمس الجديع



- Aortoiliac atheromatous calcification.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- No abdominal or pelvic collections.
- Small fat containing umbilical and right inguinal hernias are still noted.

The revised bone window revealed:

- **Rather, stationary course of the previously seen of the wide spread predominantly sclerotic osseous lesions involving the axial and appendicular skeleton.**
- **Still noted unchanged:**
 1. sternal lytic lesion with cortical disruption and thin soft tissue sheet
 2. L3 vertebral body mild anterior wedging.
 3. Multiple bilateral rib fractures , of different chronological ages
 4. Spondylo-degenerative changes.

OPINION:

Status follow up showing:

- **Stationary course regarding:**

1. **Left breast malignant mass lesion.**
2. **Wide spread predominantly sclerotic osseous metastases.**
- **No newly developed lesions.**

Reported By,

Dr. Sally Mahmoud, MSc

Dr. Hekmat Samir, MD

25-Aug-2026 03:09 PM



PATIENT NAME: NAWAL SHABAN MAKHLOF NEGEM

PATIENT ID: 2023216380

GENDER: Female

AGE: 74 y

DATE : 03-May-2026 11:32 AM

ORDER: 3853993

99mTc-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:
Compared to previous bone scan dated 10/1/2025, the current study revealed more or less stationary course of the previously seen metastatic bone deposits with more sclerotic nature of some of the lesions due to therapy effect.
Degenerative changes are seen over dorso-lumbar vertebrae.
Arthritic changes are seen over both knees.
Both kidneys are normally visualized.

Conclusion:

More or less stationary course.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant

03-May-2026 04:50 PM

PATIENT: NAWAL SHABAN MAKHLOF NEGEM, 2023216380, Female, 74 y

Report version 1, 03-May-2026 04:50 PM

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طبيب أشعة العظام
أ.د. هuda فathy، أستاذة الطب النووي، أستاذة
مستشفى جامعة القاهرة
عنوان: مستشفى جامعة القاهرة، كورنيش النيل
القاهرة - مصر

